

Transfer and Discharge Procedures

Compliance with 489.24 Regulations

Regulation: 42 CFR §489.24 | §482.13(b)(2)

POLICY OVERVIEW

Transfer and discharge procedures ensure continuity of care while protecting patient rights and meeting EMTALA obligations. These policies govern both voluntary transfers requested by patients and medically indicated transfers to appropriate levels of care.

KEY REQUIREMENTS

1 Transfer Criteria & Authorization

Transfers must be medically appropriate. A physician must certify that the benefits of transfer outweigh the risks. The patient must be stabilized to the extent possible within the facility's capabilities prior to transfer.

2 Receiving Facility Acceptance

The transferring hospital must confirm that the receiving facility has the capability and capacity to treat the patient and has agreed to accept the patient prior to transfer initiation.

3 Transfer Documentation

A copy of the medical record (or relevant portions) must accompany the patient. This includes: vital signs, assessment findings, treatments provided, labs/imaging, and reason for transfer.

4 Patient Notification & Rights

Patients must be informed of: the reason for transfer, the name of the receiving facility, their right to refuse transfer (with documentation of risks of refusal), and their rights at the receiving facility.

5 Discharge Planning

Discharge planning must begin at admission. The plan must be patient-centered, involve the patient/family, address post-acute care needs, include follow-up appointments, and provide education on medications and warning signs.

COMPLIANCE NOTES

Inappropriate transfers violate EMTALA and can result in significant penalties. The hospital must retain records of all transfers for five years. Patient advocates should be involved when patients or families express concerns about discharge plans.

COMMON SURVEY CITATIONS

Cited under §489.24(e) – Restricting transfer without stabilization, and §482.43 – Discharge planning requirements.